

STOP. Don’t Sign Anything. Read This First.

A free resource for anyone facing aged care decisions under pressure

Before you walk into any meeting, talk to the people who matter. The GP who knows your family member's history. The family members who will share this journey. And the person at the centre of it all — because however overwhelming this feels for you, it is their life, their home, and their future being decided. Make sure their voice is in the room.

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AGED CARE
GUIDE

WHO YOU'LL BE TALKING TO

During aged care decisions, you'll interact with several different people. Here's who they are and what they do:

🏥 **HOSPITAL SOCIAL WORKER / DISCHARGE PLANNER**
Works for the hospital. Their job is to coordinate your discharge. They may be helpful, but their priority is freeing up the hospital bed.

📋 **AGED CARE ASSESSOR (ACAT/ACAS)**
Government-funded. Assesses what level of care you need (home care or residential). They don't recommend specific facilities.

🏠 **AGED CARE FACILITY STAFF**
Work for the facility. They'll explain costs and show you around. Remember: they want you to choose their facility.

💰 **FINANCIAL ADVISER**
Optional, but valuable. A fee-only aged care financial adviser can help you understand costs and asset protection. Make sure they're "fee-only" (you pay them directly, not commission-based).

👴⚖️ **AGED CARE PLACEMENT SERVICE**
Some families use these. They help find suitable facilities. Check if they charge you or get commissions from facilities (conflict of interest).

Each person has different knowledge and different motivations. That's why you ask different questions to different people.

IMPORTANT NOTE

This information is based on Australian Government aged care regulations, which apply nationally. Some details (wait times, facility availability, costs, some state support programs) may vary by state or region. The assessment services are called ACAT in most states and ACAS in Victoria—same function, different name.

Print the next 6 pages. Take them to every meeting. Ask all 6 questions.

QUESTION 1

WHO TO ASK: The Hospital Social Worker or Discharge Planner

"What happens if we need more time to decide?"

Why it matters: Hospitals want their bed back. They may pressure you to decide today. But most aged care decisions can wait 48-72 hours without any medical risk.

Ask them: "What is the medical deadline for discharge, and what happens if we need 48 more hours to research our options?"

What a good answer sounds like:
"Your [parent/spouse] is medically stable. Discharge isn't urgent. You can take a few days. We can arrange short-term respite care while you explore your options."

Red flag answers:
"You need to pick a facility today or we can't help you."
"The only bed available is at [specific facility] and you need to decide now."

What this question reveals is whether the urgency is real (medical) or artificial (administrative). If it's artificial, you have time to make better decisions.

QUESTION 2

WHO TO ASK: The Hospital Discharge Planner or Aged Care Assessor

"Is residential aged care our only option, or are there home care options we haven't explored?"

Why it matters: Hospitals push residential care because it's the fastest way to free up the bed. But many people can safely stay at home with proper support. Rushing into residential care often means you move twice: first to a facility that's available NOW, then to a better one later. Or you realize home care would have worked and you moved too soon.

Ask them: "Has [parent/spouse] been assessed for home care packages? What level of home care are they eligible for, and what would that actually look like day-to-day compared to residential care?"

"Can we try home care first, or do short-term respite care, and move to residential later if home care doesn't work?"

What a good answer sounds like:
"Yes, they've been assessed for a Level 4 home care package. This would provide:

- Daily visits from a support worker (personal care, meals, medication)
- Weekly nursing visits
- Allied health services (physio, occupational therapy)
- Up to $55,000/year in government-funded support

Many people with similar needs do well at home with this level of support. The trade-off is:

- Home care: They stay at home, more independence, but you coordinate the services
- Residential care: 24/7 supervision, less coordination for you, but less independence for them

Red flag answers:
"Home care won't work for them."
"Residential care is the only safe option."
Presenting residential care as urgent without mentioning respite care as a trial option.
Not mentioning that they're eligible for home care packages, or acting like home care doesn't exist.

What this question reveals: Whether they're actually assessing ALL your options or just pushing the fastest solution for the hospital. Good discharge planners present options. Bad ones present one path and pressure you to take it.

QUESTION 3

WHO TO ASK: The Aged Care Facility You're Considering

"What does your nursing coverage look like overnight and on weekends?"

Why it matters: Staffing is where care quality lives or dies. A facility can look beautiful in a tour and be dangerously understaffed at 2am. Minimum staffing requirements exist, but meeting the minimum and providing good care are not the same thing.

Ask them: "How many registered nurses are on duty overnight? Is there always an RN physically on site, or are they on-call? What's the staff-to-resident ratio on weekends compared to weekdays?"

What a good answer sounds like: "We have a registered nurse on site 24 hours a day, 7 days a week. Weekend ratios are the same as weekdays. I can show you our current staffing roster."

Red flag answers: "We meet all government requirements." (Minimum compliance is not the same as good care — push for actual numbers) "We have a nurse on call overnight." (On-call means not physically present — residents wait) Hesitation, vagueness, or refusing to show you a roster.

What this question reveals is whether the facility is genuinely transparent about care quality, not just costs. Good facilities are proud of their staffing. Ones that aren't will change the subject.

QUESTION 4

WHO TO ASK: The Aged Care Facility You're Considering

"What are ALL the ongoing costs we'll pay every month, not just the entry fee?"

Why it matters: The big upfront payment (RAD) gets all the attention. But you'll also pay monthly fees FOREVER. These can be $2,000-$5,000+ per month. Families don't realize this until after they've signed.

Ask them: "Can you give me a written breakdown showing every single fee we'll pay each month? I need to see:

- The basic daily care fee
- The means-tested care fee (based on our income/assets)
- Any extra service fees
- Any other ongoing charges
- What will our total monthly cost be, based on our financial situation?"

What a good answer sounds like:
They sit down with you and show you a written estimate like this:
"Based on what you've told me about your income and assets, here's your estimated monthly cost:
Basic daily fee: $64 per day = ~$1,950/month
Means-tested care fee: ~$1,150/month (based on preliminary assessment)
Extra service fee: $35/day = ~$1,070/month
Total estimated monthly cost: ~$4,170/month
I'm giving you this in writing. Your means-tested fee might change slightly after your formal assessment, but this estimate should be close."

Red flag answers:
"It's about $60 a day."
"We can't estimate costs until you move in."
"It depends on your assessment.
"Just pay the RAD and don't worry about monthly fees."

What this question reveals is whether the facility is being transparent about the real, ongoing cost. If they won't put it in writing, they're hiding something.

QUESTION 5

WHO TO ASK: The Aged Care Assessor (ACAT) or a Fee-Only Financial Adviser

"What happens to our family home in the means test?"

Why it matters: This is the $80,000-$200,000 mistake families make. Many people sell their house immediately to pay for aged care, not knowing the house might be EXEMPT from the asset test for up to 2 years. Selling too early means you pay capital gains tax, lose the exemption period, and can't benefit if property values rise.

Ask them: "Is our family home exempt from the aged care means test? For how long? And if we sell the house now versus waiting 12-24 months, how does that change what we pay for aged care?"

What a good answer sounds like:
Scenario 1 - Spouse or adult child is still living at home: "Your home is fully exempt as long as your spouse lives there. It won't count as an asset. Don't sell it. When your spouse passes away or moves into care, the rules change—but for now, keep it."

Scenario 2 - Both entering care, no one at home: "Your home is exempt for up to 2 years from the date you enter care. During this time, it doesn't count as an asset for the means test. You can rent it out, leave it vacant, or sell it—but if you sell it within those 2 years, the sale proceeds become an asset and will increase your fees. Consider waiting until near the end of the exemption period before selling, or talk to a financial adviser about whether selling makes sense for you."

Red flag answers:
"Just sell the house to pay for care."
"The house counts as an asset, so you'll need to sell it."
Refusing to discuss this or saying "it's too complicated."

Important: This is the question where you MOST need professional advice. A fee-only financial adviser who specializes in aged care can save you six figures. Don't trust free advice from facilities—they may have conflicts of interest.

What this question reveals is whether the person understands aged care means testing rules. If they don't mention exemptions, they don't know the system well enough to be advising you.

QUESTION 6

WHO TO ASK: The Aged Care Facility

"Can we leave if this isn't the right fit, and what does it cost to exit?"

Why it matters: Some families realize within weeks that they've chosen the wrong facility (wrong level of care, poor quality, personality clash, etc.). You need to know: Can you leave? How much will it cost? How long does it take to get your money back?

Ask them: "What is your exit policy? If we decide to move [parent/spouse] to a different facility or back home:

- Are there any exit fees?
- Is there a minimum stay requirement?
- How long does it take to refund our accommodation payment (RAD)?
- Do we get all of it back, or are there deductions?"

What a good answer sounds like:
“Here's a copy of our exit policy in writing."
"You can leave anytime—there's no minimum stay. Here's how the refund works:
If you paid a lump sum RAD of $650,000:

- We refund the full $650,000 (no deductions or 2% PA capped at 5 years))
- Timing: We refund within 14 days of the room being filled by a new resident, or 6 months, whichever comes first
- There are no exit fees

If you've been paying a daily accommodation payment (DAP) instead of a lump sum, you just stop paying when you leave—nothing to refund.

Red flag answers:
"No one ever leaves once they're here."
"There's an exit fee of [amount]."
"You'll get your money back eventually."
Refusing to provide exit policy in writing.

What this question reveals is whether this decision is truly reversible or if you're locked in. Good facilities are confident you'll stay because they provide good care—they're not trying to trap you.

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WHAT TO DO WITH THESE QUESTIONS

1. Print this guide
   Take it to every meeting. Don't rely on memory.

2. Ask all 6 questions
   Don't skip any. They each reveal different critical information.

3. Write down the answers
   Document who said what. You'll need this later.

4. Compare answers
   If different people give conflicting answers, that's a red flag.

5. Trust your gut
   If someone dodges these questions or pressures you, find someone else.

REMEMBER:

✓ You don't have to decide everything today
✓ Asking questions is not rude—it's responsible
✓ Good professionals welcome these questions
✓ Pressure tactics are red flags
✓ Some decisions are reversible; some are not

The questions you ask now can save you six figures.

NEED MORE THAN JUST QUESTIONS?

These 6 questions will help you avoid the biggest mistakes.

But if you're facing major aged care decisions and want the full picture that prevents costly mistakes, get the complete aged care guide:
- How costs actually work
- Every major fee explained
- Home care vs residential trade-offs
- What can wait and what cannot
- Printable checklists for every decision

"Before You Sign: The Aged Care Decision Guide" The complete reference Australian families keep and share

Learn more at: www.agedcaredecisionguide.com

DISCLAIMER

This guide provides general information only and does not constitute financial, legal, or medical advice. Aged care rules are complex and individual circumstances vary. While we have made every effort to ensure accuracy, aged care regulations may change after publication. Always verify current rules and seek professional advice before making decisions that affect your financial or care situation.

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